

GENERALIZED HYPERTRICHOSIS

There are several drugs that routinely cause generalized hypertrichosis. Oral minoxidil, a piperidinopyrimidine derivative and potassium channel opener used as a systemic antihypertensive agent, causes hypertrichosis in 80% of patients, most prominently on the face, shoulders, and extremities. Diazoxide, a benzothiadiazine used in malignant hypertension or idiopathic hypoglycemia of infancy, leads to lanugo-like hypertrichosis of the face, trunk, and extremities in 1% to 20% of adults, but in nearly 100% of children. Phenytoin (Dilantin) use results in terminal hair hypertrichosis in 5% to 12% of patients, typically beginning on the extremities, trunk, and face. The immunosuppressive immunophilin ligand cyclosporine A causes terminal hair hypertrichosis in 40% to 95% of patients, with a diffuse increase in hair, primarily over the upper body. Based on findings in mouse models, the underlying mechanism likely involves premature induction of the anagen phase and inhibition of catagen by cyclosporine A, which may contribute to the conversion of vellus to terminal hair, resulting in hypertrichosis.

LOCALIZED HYPERTRICHOSIS

Table 86-11 lists conditions associated with localized hypertrichosis, which are generally inherited, developmental, or secondary to irritation or trauma.

Causes of Localized Hypertrichosis

Congenital/Hereditary

- Mid-dorsum hypertrichosis with underlying neuroectodermal abnormality
- Spina bifida occulta
- Traction (e.g., meningocele manqué)
- Diastematomyelia
- Familial cervical hypertrichosis
- Hairy elbows
- Anterior cervical hypertrichosis
- Hairy pinna
- Trichomegaly alone
- Congenital smooth muscle hamartoma
- Winchester syndrome
- Nevoid hypertrichosis
- Stiff skin syndrome
- Congenital pigmented nevus
- Hemimaxillofacial dysplasia
- Becker nevus

Acquired

- Melorheostotic scleroderma
- Post morphea
- Reflex sympathetic dystrophy
- Osteosclerotic myeloma
- Scrotal hair in male infants
- Drugs
- Interferon
- Sodium tetradecyl sulfate
- Topical latanoprost

- Topical minoxidil
- Irritants (topical)
- Repeated trauma
- Pressure (e.g., sack bearers, costaleros)
- Lichen simplex chronicus
- Biting
- Insect bites
- Stasis/lymphedema
- Congenital arteriovenous fistula
- Denervated area
- Chronic osteomyelitis
- Site of immunizations
- Smallpox
- Diphtheria-tetanus
- Chicken pox
- Human immunodeficiency virus
- Periphery of burned area
- Kala-azar (eyelashes)

TREATMENT

Removal of the inciting cause should be the primary approach to treatment and usually leads to regression of hypertrichosis. When this is not possible, management focuses on helping the patient cope psychologically with the physical change and/or using temporary or permanent hair removal methods. The same techniques used for hirsutism management can be applied for hypertrichosis, except for anti-androgenic therapies.